

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	Linda Cho
Date of birth	April 3, 1968
Age at visit	58 years
Sex	Female
MRN	GSV-45110
Address	640 Channelside Drive, Apt 12, Tampa, FL 33602
Phone	(813) 555-0188
Emergency contact	Michael Cho (brother), (727) 555-0144
Primary care	Dr. Anita Shah, MD, (813) 555-0190
Preferred language	English

2. Insurance

Primary insurance

Payer	Florida Blue, BlueOptions PPO
Member ID	XFB551902776
Group	FL-22910
Prior authorization	Required for vein ablation
Auth telephone	1-800-555-0194

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I83.812	Varicose veins of left lower extremity with pain	2025	Active
I80.03	Phlebitis and thrombophlebitis of superficial vessels of lower extremities, bilateral	2025	Active
I87.2	Chronic venous insufficiency (peripheral)	2025	Active

4. Current medications

Medication	Dose	Frequency	Indication
Naproxen	220 mg	As needed	Leg pain after phlebitis
Compression stockings	20-30 mmHg knee-high	Daily while upright	Venous insufficiency

5. Allergies

Allergen	Reaction	Severity
Latex	Localized hives	Mild

6. Visit notes — vascular surgery

Visit date: January 8, 2026 | **Attending:** Elena Vasquez, MD, FACS | **Type:** New patient consult

Chief complaint. Recurrent superficial thrombophlebitis of the left calf and persistent aching.

History of present illness. Linda Cho is a 57-year-old teacher. She had superficial thrombophlebitis of a left calf varicosity in March 2025 and a second, separate episode in July 2025. Both were documented by her primary physician, treated with naproxen and warm compresses, and resolved. Since July the left leg still aches when she stands to teach. The aching significantly interferes with activities of daily living because she cannot stand through a full class period. She has no ulcer and no bleeding. She has not had a deep-vein thrombosis. She is willing to try stockings but wants a procedure eventually. Radiofrequency ablation and mechanochemical ablation were both described. She asked questions about ClariVein and did not choose a modality today.

Examination.

Blood pressure 128/76 mmHg	Left leg Residual cord of the July phlebitis. Visible varicosities. No ulcer. No acute erythema today.
CEAP C2, symptomatic	Right leg Small telangiectasias only. No axial varicosities.

Assessment and plan.

Two episodes of superficial thrombophlebitis in the left leg, March 2025 and July 2025, plus ongoing pain that interferes with teaching.

Order left venous duplex.

Fit 20-30 mmHg knee-high stockings today, January 8, 2026, and wear them daily.

Electronically signed: Elena Vasquez, MD, FACS — January 8, 2026 at 10:15 AM

Visit date: April 14, 2026 | **Attending:** Elena Vasquez, MD, FACS | **Type:** Follow-up

Chief complaint. Stockings have not helped. She wants mechanochemical ablation.

History of present illness. She wore 20-30 mmHg knee-high stockings daily from January 8, 2026 through April 14, 2026, fourteen weeks. Fourteen weeks of compression therapy has not improved the aching, and the symptoms still significantly interfere with activities of daily living. There has been no third episode of phlebitis during the stocking trial. There is no ulcer and no bleeding. She was offered endovenous radiofrequency ablation and declined it. She requests mechanochemical ablation with a ClariVein-type catheter.

Examination.

Compression Daily 20-30 mmHg from January 8, 2026 through April 14, 2026	Symptom change Aching unchanged
Skin No ulcer. Old phlebitic cord is nontender.	CEAP C2, symptomatic

Assessment and plan.

Left great saphenous reflux and diameter are confirmed on the January 20, 2026 duplex. CEAP C2.

Indications present: two prior episodes of superficial thrombophlebitis, and pain that interferes with daily activities after fourteen weeks of compression.

Order mechanochemical ablation of the left great saphenous vein, CPT 36473. She declined radiofrequency ablation.

Electronically signed: Elena Vasquez, MD, FACS — April 14, 2026 at 1:44 PM

7. Diagnostic reports

Venous duplex reflux study — January 20, 2026

Ordering physician	Elena Vasquez, MD, FACS
Facility	Gulfshore Vascular Lab
Indication	Recurrent superficial thrombophlebitis, left leg
Technique	Upright duplex

Interpretation. Left great saphenous vein: reflux lasting 2.0 seconds at the saphenofemoral junction. Great saphenous diameter 8.1 millimeters in the proximal thigh. The previously inflamed tributary is patent and compressible. Small saphenous vein is competent. Deep veins are patent. No pathologic perforator. Impression: left great saphenous reflux, diameter 8.1 millimeters, in a limb with prior superficial thrombophlebitis.

Electronically signed: James Whitfield, MD, RPVI — January 20, 2026

8. Planned procedure

Planned procedure	Mechanochemical ablation (ClariVein-type catheter) of the left great saphenous vein
CPT	36473 — mechanochemical ablation, first vein
Laterality	Left leg
Vein	Great saphenous vein only
Surgeon	Elena Vasquez, MD, FACS
Declined alternative	Radiofrequency ablation was offered and declined
Payer	Florida Blue BlueOptions PPO
Authorization status	Not submitted. Request dated April 14, 2026.

Clinical indication summary

Left great saphenous reflux 2.0 seconds and diameter 8.1 millimeters on January 20, 2026.

CEAP C2, symptomatic.

Superficial thrombophlebitis of the left leg in March 2025 and again in July 2025.

Pain significantly interferes with activities of daily living.

Daily compression from January 8, 2026 through April 14, 2026 (fourteen weeks) has not improved symptoms.

The ordered code is 36473, mechanochemical ablation, not radiofrequency or laser.

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